

YATHARTH PATIENT GUIDE

ANTERIOR CERVICAL DISCECTOMY AND FUSION (ACDF) SURGERY

Guide

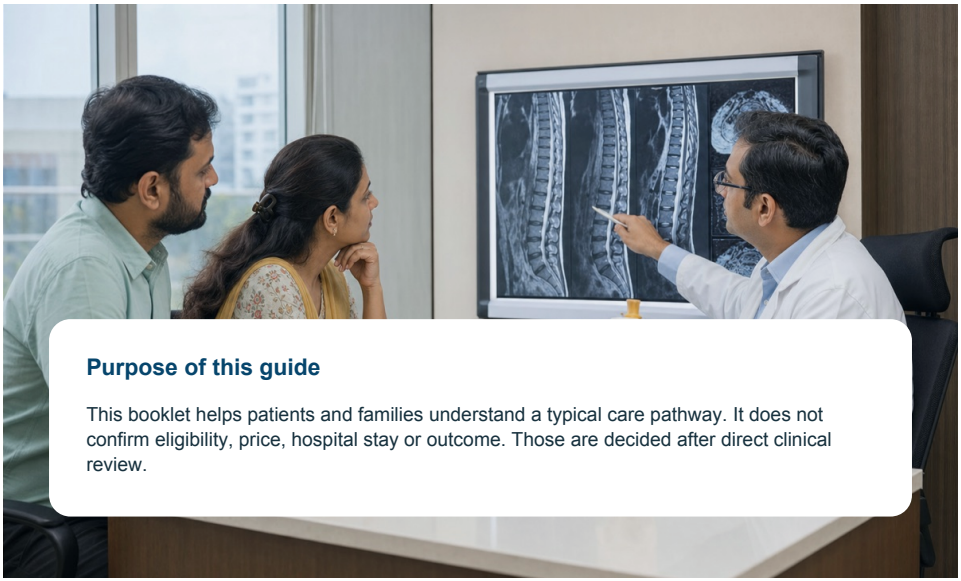
Evaluation, procedure, recovery, international patient planning and package questions.

Yatharth Super Speciality Hospitals | September 2026

INTRODUCTION

Anterior Cervical Discectomy and Fusion (ACDF) Surgery

Neck surgery that removes a damaged disc and fuses selected cervical levels to relieve nerve or spinal-cord pressure.



Purpose of this guide

This booklet helps patients and families understand a typical care pathway. It does not confirm eligibility, price, hospital stay or outcome. Those are decided after direct clinical review.

TREATMENT OVERVIEW

When it may be considered

A specialist confirms the diagnosis and whether this treatment is appropriate.

Common indications

- Cervical disc herniation with nerve compression
- Spinal-cord compression or myelopathy
- Persistent arm pain or weakness
- Failure of appropriate non-surgical care

Treatment types or pathways

- Single-level or multi-level ACDF
- Graft or cage with plate fixation
- Alternative cervical procedures when more suitable

Important

The presence of an indication does not mean treatment is automatically suitable. Age, anatomy, disease stage, previous treatment, other illnesses and patient preference all matter.

MEDICAL EVALUATION

Tests and specialist review

The team may add or remove tests based on the patient's condition and proposed treatment.

Typical evaluation

- Standing X-rays or MRI as appropriate
- Blood counts, kidney function and clotting tests
- Heart and anaesthesia assessment
- Infection and dental review when relevant
- Mobility, home and rehabilitation assessment

Always bring

- Recent doctor notes and diagnosis
- Reports with dates and units
- Imaging reports plus DICOM files when available
- Current medicines, allergies and prior procedures
- Passport details only when required for international-patient processing

BEFORE TREATMENT

Preparation and admission

Follow only the individualized written instructions issued by the treating and anaesthesia teams.

Preparation checklist

- Pain, function and non-surgical treatments reviewed
- Implant or fixation plan discussed
- Blood-thinner and diabetes instructions confirmed
- Walking aid and caregiver arranged
- Physiotherapy goals set before admission

Medicines and fasting

Do not stop blood thinners, diabetes medicines, steroids, seizure medicines or other prescribed treatment unless the responsible clinician gives specific instructions.

Tell the team promptly

Report fever, infection, new breathing trouble, chest pain, pregnancy possibility, recent hospital admission or any major change before travelling or admission.

TREATMENT PLANNING

How the team selects a pathway

Clinical decisions balance likely benefit, risk, alternatives and the patient's goals.

Key decision factors

- Pain and loss of function
- Imaging findings and joint or tissue damage
- Response to medicines, injections and physiotherapy
- Age, activity goals and medical fitness
- Ability to participate in rehabilitation

Questions to ask

- What is the goal of this treatment?
- What alternatives are reasonable?
- What result is realistic for this patient?
- What are the major risks and warning signs?
- What would change the plan or total estimate?

PROCEDURE

How it is usually performed

The final technique, anaesthesia and devices are selected by the treating team.

1. Neck approached from the front
2. Disc and compressive material removed
3. Cage or graft placed
4. Fusion stabilized with selected fixation

Technique may change

Safety findings during treatment can require a different device, additional procedure, conversion to open surgery, longer monitoring or referral to another specialist.

AFTER TREATMENT

Recovery, monitoring and follow-up

Discharge depends on clinical milestones, not only a planned number of days.

Monitoring

- Pain, wound and swelling
- Walking or movement milestones
- Blood-clot prevention
- Physiotherapy progression
- Follow-up imaging when indicated

Possible risks

- Infection, bleeding or blood clots
- Stiffness or persistent pain
- Nerve or vessel injury
- Implant, fixation or healing problems
- Need for further surgery

Seek urgent help

Use the hospital's emergency route or nearest emergency service for severe pain, breathing difficulty, heavy bleeding, fainting, new weakness, persistent vomiting, confusion, high fever or rapidly worsening symptoms.

INTERNATIONAL PATIENTS

Documents to prepare

The hospital team must verify country-specific and treatment-specific requirements before travel.



Travel documents

- Valid passport for patient and attendant
- Correct medical and attendant visas
- Hospital invitation or support letter
- Recent medical reports in English
- Travel and medical insurance details

Clinical documents

- Diagnosis and treatment summary
- Imaging files and pathology material if requested
- Current prescriptions and allergy list
- Emergency contact and caregiver details
- Transplant relationship and authorization documents when applicable

Protect personal information

Share passports and medical records only through the Yatharth channel confirmed for the case. Do not send passwords, payment PINs or unrelated identity documents.

PACKAGE PLANNING

Hospital stay and inclusions

Outpatient procedures may need brief observation; joint replacement or spine surgery usually requires admission and rehabilitation before travel.

Confirm each inclusion in writing

- Treating-team fees specifically listed
- Procedure or test named in the estimate
- Defined room or ICU category and included days
- Routine medicines and consumables listed
- Named implant, device, blood product or special medicine
- Included investigations and follow-up consultations

A provisional estimate is not a final bill

Changes in diagnosis, medical condition, technique, implant, ICU need or length of stay can change the total. Ask who can approve a revised estimate.

PACKAGE PLANNING

Common exclusions or additional charges

Exact exclusions depend on the written estimate issued for this patient.

Check before payment

- Stay beyond the included room or ICU days
- Treatment of complications or unrelated conditions
- Cross-consultations not named in the package
- High-cost medicines, implants or devices not listed
- Additional procedures, investigations or pathology
- Post-discharge medicines, tests or rehabilitation
- Travel, accommodation and companion expenses
- Applicable taxes or payment-processing charges

Keep the estimate, receipts, consent forms, implant labels, discharge summary and final itemized bill.

PLANNING YOUR VISIT

Financial, visa and accommodation guidance

Confirm arrangements with Yatharth's International Patient Services before booking non-refundable travel.

Payment

- Confirm currency and estimate validity
- Use hospital-approved payment routes
- Keep proof of every payment

Visa

- Request the correct hospital support letter
- Use the visa category required for treatment
- Check extension requirements early

Accommodation

- Choose a stay near the treating hospital
- Confirm accessibility and caregiver needs
- Plan extra days for unexpected review

Language and food

- Request interpreter support in advance
- Share dietary or religious requirements
- Follow the clinical diet after treatment

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Treatment decisions and estimates require direct clinical review.